

AP PSYCHOLOGY · THE UNIVERSAL STUDY GUIDE

Targeting a 3, 4, or 5 — every concept tagged so you know what your tier needs

Tier badges: [3] foundational · [4] adds for 4+ · [5] adds for 5

Targeting a 4 → study [3] AND [4] items. Targeting a 5 → study all three tiers.

PAGE 1 | MATH OF 3/4/5 · 5 UNITS · THE 5 PERSPECTIVES

About this exam. AP Psychology was redesigned for 2024-25, consolidating from 9 units to 5 broader units, with greater emphasis on research methods and scientific argumentation. The exam is heavy on terminology recognition and applying concepts to scenarios.

THE MATH OF YOUR TARGET

Target	MCQ (75 q, 67%)	FRQ (2 q, 33%)	Approx %	Tier focus
3 (PASS)	~38 / 75 (~51%)	Earn ~7-8 / 14 raw	~50%	[3] only
4 (SOLID)	~46 / 75 (~61%)	Earn ~9-10 / 14 raw	~62%	[3] + [4]
5 (MASTERY)	~56 / 75 (~75%)	Earn ~11-12 / 14 raw	~75%	All three

THE 5 UNITS — by exam weight (post-2024 redesign)

#	Unit	Weight	Key topics
1	Biological Bases of Behavior	15–20%	Neurons, brain structure, neuroplasticity, sleep, sensation
2	Cognition	20–25%	Memory, perception, thinking, intelligence, language
3	Development and Learning	20–25%	Classical/operant conditioning , lifespan development, social cognition
4	Social Psychology and Personality	15–20%	Conformity, attitudes, attribution, personality theories
5	Mental and Physical Health	20–25%	Disorders, treatments , stress, positive psychology

Units 2, 3, and 5 each command 20-25% of the exam. Cognition (memory!), conditioning, and disorders are the heart of AP Psych.

THE 5 PSYCHOLOGICAL PERSPECTIVES — every concept fits one (or more) [3]/[4]

Perspective	Core idea	Example
Biological [3]	Behavior arises from biology — genes, neurons, hormones, brain structure	Depression linked to low serotonin
Cognitive [3]	Behavior arises from thought processes — attention, memory, perception, problem-solving	Anxiety from catastrophic thinking patterns
Behavioral [3]	Behavior is learned through conditioning — environmental rewards/punishments shape us	Phobias from classical conditioning
Sociocultural [4]	Behavior shaped by culture, society, and group membership	Conformity differs across cultures (individualist vs collectivist)
Humanistic [4]	Emphasizes free will, self-actualization, personal growth, and the conscious experience	Maslow's hierarchy of needs
Psychodynamic [4]	Behavior shaped by unconscious drives and early childhood experiences (Freud)	Defense mechanisms hide threatening thoughts
Evolutionary [5]	Behavior shaped by natural selection — adaptive value of traits	Fear of snakes is evolutionarily adaptive

KEY INSIGHT [5] On FRQs, name the perspective explicitly. Saying "From a **biological perspective**, depression is linked to low serotonin levels..." earns more credit than "depression is linked to low serotonin." The exam rewards perspective-tagging.

PAGE 2 | RESEARCH METHODS · the exam now emphasizes this heavily

The 2024 redesign brought research methods to the FORE. Both FRQs (AAQ and EBQ) require analyzing scientific articles. You must understand variables, study designs, statistics, and ethical principles cold.

Types of Research [3]/[4]

Method	Description	Strength / Limitation
Experiment [3]	Manipulate IV, measure DV, random assignment to conditions	Only method that establishes causation
Correlational study [3]	Measure relationship between two variables (no manipulation)	Doesn't establish causation. r ranges from -1 to $+1$.
Case study [3]	In-depth study of single individual or small group	Rich detail; can't generalize
Naturalistic observation [3]	Watching behavior in natural setting without intervention	High ecological validity; can't control variables
Survey [3]	Self-report questionnaires	Cheap and broad; vulnerable to social desirability bias
Longitudinal [4]	Same group studied over many years	Tracks development; participant attrition
Cross-sectional [4]	Different age groups studied at one time	Faster but cohort effects can confound
Meta-analysis [4]	Statistical combination of many studies	Powerful for estimating effect size

Variables in Experiments [3]/[4]

IV (independent variable) = what the researcher MANIPULATES
 DV (dependent variable) = what the researcher MEASURES
 CONFOUNDING variable = uncontrolled factor that could explain results
 OPERATIONAL definition = precise description of how a concept is measured
 ("happiness measured by 1-10 self-report scale")

CONTROLS:

Random sampling – generalize to population
 Random assignment – establish causation; balance groups
 Control group – comparison baseline
 Single-blind – participants don't know condition
 Double-blind – neither participants nor experimenters know
 Placebo – inert treatment used as control

Statistics — the basics [3]/[4]

- [3] **Mean, median, mode** — measures of central tendency. Skewed data: median is more robust.
- [3] **Standard deviation** — measure of spread. Larger SD = more variable data.
- [4] **Statistical significance ($p < 0.05$)** — result unlikely due to chance alone. Doesn't prove the hypothesis, just makes it more credible.
- [4] **Effect size** — magnitude of effect, separate from significance. Big sample can find statistically significant tiny effects.
- [4] **Correlation coefficient (r)** — strength + direction of linear relationship. $r = +1$ perfect positive, 0 none, -1 perfect negative. $r \neq$ causation.
- [5] **Normal distribution** — bell curve. ~68% within 1 SD, 95% within 2 SDs, 99.7% within 3 SDs.

Ethics in Research [4]

- [3] **Informed consent** — participants know what they're agreeing to before participating.
- [3] **Confidentiality** — data anonymized.
- [3] **Right to withdraw** — can leave at any time.
- [4] **Debriefing** — explain study purpose afterward, especially if deception used.
- [4] **IRB (Institutional Review Board)** approval required for human subjects research.
- [4] **Animal research** — minimize harm, justify scientific value.

WATCH OUT [4] **Famous ethical violations are FRQ favorites.** Milgram's obedience study (deception, distress without proper debriefing); Stanford Prison Experiment (no early termination); Tuskegee (no informed consent, denied treatment). Be ready to identify what was wrong.

PAGE 3 | MASTER VOCABULARY · the 50 highest-frequency terms

These terms appear repeatedly across MCQ and FRQ scenarios. Tag each as you study — knowing a term in context (vs just by definition) is what gets you above a 3.

Term + tier	Definition / how it's tested
Confirmation bias [3]	Tendency to seek + remember evidence that confirms existing beliefs.
Hindsight bias [3]	"I knew it all along" effect — overestimating predictability after the fact.
Heuristics [4]	Mental shortcuts. Availability (judging by what comes to mind); Representativeness (judging by similarity to typical case).
Cognitive dissonance [3]	Discomfort from holding conflicting beliefs/behaviors. We resolve by changing one.
Conformity [3]	Adjusting behavior/beliefs to match a group. Asch line studies are the classic example.
Obedience [3]	Following orders from authority. Milgram shock studies showed 65% obedience to harmful commands.
Bystander effect [3]	Less likely to help when others present. Diffusion of responsibility.
Fundamental attribution error [3]	Explaining OTHERS' behavior by their personality, ignoring situation. (We attribute our own behavior to situation.)
Self-serving bias [4]	Attributing successes to self, failures to circumstances.
Schema [3]	Mental framework for organizing information. Schemas shape perception and memory.
Working memory / short-term [3]	Limited capacity (~7±2 items), short duration (~30 sec). Chunking expands capacity.
Long-term memory [3]	Vast capacity, long-lasting. Explicit (declarative) : facts + events. Implicit (procedural) : skills + classical conditioning.
Encoding / storage / retrieval [3]	The 3 stages of memory. Failure at any stage = forgetting.
Misinformation effect [4]	Memory distorted by misleading post-event information (Loftus eyewitness studies).
Classical conditioning [3]	Pavlov. UCS → UCR; pair with NS until NS → CR. Key: stimulus PRECEDES response .
Operant conditioning [3]	Skinner. Behavior shaped by consequences. Reinforcement strengthens; punishment weakens.
Reinforcement: positive vs negative [4]	Positive = ADD desirable (treat). Negative = REMOVE aversive (seatbelt alarm stops). Both INCREASE behavior.
Schedules of reinforcement [4]	Fixed-ratio, variable-ratio, fixed-interval, variable-interval. Variable schedules → most resistant to extinction (e.g., gambling).
Observational learning [3]	Bandura's Bobo doll: kids imitate aggression seen in adults.
Algorithm vs heuristic [4]	Algorithm: step-by-step, guaranteed solution. Heuristic: shortcut, may fail.
Fixation / functional fixedness [4]	Inability to see new uses for familiar objects.
Stages of sleep [3]	REM (rapid eye movement, dreams, paralysis); NREM (3 stages, deep sleep). Cycles ~90 min .
Circadian rhythm [3]	~24-hour cycle, regulated by SCN + melatonin.
Sensation vs perception [3]	Sensation: detection by sense organ. Perception: interpretation by brain.
Top-down vs bottom-up processing [4]	Top-down: schemas/expectations shape perception. Bottom-up: raw sensory data builds up.
Absolute threshold [3]	Minimum stimulus detected 50% of the time.
Just-noticeable difference (JND) [4]	Smallest detectable difference. Weber's law : JND is proportional to original intensity.
Big Five personality (OCEAN) [4]	Openness, Conscientiousness, Extraversion, Agreeableness, Neuroticism.
Maslow's hierarchy [3]	Physiological → Safety → Love/Belonging → Esteem → Self-actualization.
Defense mechanisms [4]	Freud: repression, denial, projection, rationalization, sublimation, displacement.
Erikson's stages [4]	8 psychosocial stages. Each has crisis: trust vs mistrust (infancy), identity vs role confusion (adolescence), etc.
Piaget's stages [4]	Sensorimotor (0-2), Preoperational (2-7), Concrete operational (7-11), Formal operational (12+).
Authoritarian / authoritative / permissive [4]	Parenting styles. Authoritative (warm + firm) → best outcomes.
Cognitive dissonance [4]	Festinger. Tension from holding contradictory cognitions → motivates change.
Stress: GAS [3]	Selye's General Adaptation Syndrome: alarm → resistance → exhaustion.
Anxiety disorders [3]	GAD, panic, phobias, OCD, PTSD.
Mood disorders [3]	Major depressive disorder, bipolar disorder.
Schizophrenia [4]	Positive symptoms (hallucinations, delusions); negative (flat affect, social withdrawal).
Cognitive-behavioral therapy (CBT) [4]	Combines challenging distorted thoughts with behavioral techniques. Most evidence-based therapy.
SSRIs [4]	Selective serotonin reuptake inhibitors. First-line antidepressants (Prozac, Zoloft).

UNIT 1 | BIOLOGICAL BASES OF BEHAVIOR · neurons, brain, sleep

The Neuron — communication at the cellular level [3]/[4]

STRUCTURE:

- Dendrites — receive signals
- Soma — cell body
- Axon — transmits action potential
- Myelin — insulates axon, speeds conduction
- Terminal — releases neurotransmitters

PROCESS:

1. Resting potential (~-70 mV inside)
2. Stimulation depolarizes membrane
3. Threshold reached → ALL-OR-NONE firing
4. Action potential travels down axon
5. Neurotransmitters released into synapse
6. Bind to next neuron's receptors
7. Reuptake or enzyme breakdown clears synapse

Key Neurotransmitters [3]/[4]

Neurotransmitter	Function	Disorders / drugs
Acetylcholine [3]	Muscle contraction, learning, memory	Alzheimer's (low ACh)
Dopamine [3]	Reward, movement, motivation	Parkinson's (low); schizophrenia (excess); cocaine boosts
Serotonin [3]	Mood, sleep, appetite	Depression (low); SSRIs increase availability
Norepinephrine [4]	Arousal, alertness, fight-or-flight	Anxiety (high); depression (low)
GABA [4]	Major INHIBITORY neurotransmitter	Anxiety (low); benzodiazepines enhance
Glutamate [4]	Major EXCITATORY neurotransmitter	Memory; over-excitation linked to migraines/seizures
Endorphins [3]	Natural painkillers, pleasure	Released during exercise; opioids mimic

Brain Structure — by region [3]/[4]

Region	Function
Brainstem [3]	Medulla (heart rate, breathing); Pons (sleep, arousal); Reticular formation (alertness)
Cerebellum [3]	Coordination, balance, motor learning
Thalamus [3]	Sensory relay station (all senses except smell route through here)
Hypothalamus [3]	Hunger, thirst, body temp, sex drive. Controls pituitary gland.
Limbic system [3]	Amygdala (emotion, especially fear); Hippocampus (memory formation)
Cerebral cortex (4 lobes) [3]	Frontal (executive function, motor, Broca's area); Parietal (touch, spatial); Occipital (vision); Temporal (hearing, Wernicke's area)

Brain Lateralization & Plasticity [4]/[5]

- **Left hemisphere** [4]: language (Broca's, Wernicke's), logical/sequential.
- **Right hemisphere** [4]: spatial reasoning, facial recognition, emotional expression.
- **Corpus callosum** [4] connects hemispheres. **Split-brain patients** (severed CC) reveal hemisphere-specific functions.
- **Plasticity** [3]/[4]: brain reorganizes after injury. Especially strong in young brains.
- **Neurogenesis** [5]: new neurons form throughout life (especially hippocampus).

Sleep [3]/[4]

- **4 stages** that cycle every ~90 min: NREM 1 → 2 → 3 → REM.
- **REM sleep** [3]: rapid eye movement, vivid dreams, body paralysis. ~25% of sleep.
- **NREM 3 (deep sleep)** [3]: slow-wave; restorative; growth hormone released.
- **Theories of dreams** [4]: Wish fulfillment (Freud); Information processing/memory consolidation; Activation-synthesis (random brain activity interpreted as story).

UNIT 2 | COGNITION · memory, perception, thinking, intelligence

The Memory System — the 3-stage model [3]

SENSORY MEMORY (~few seconds)
 Iconic (visual, ~0.5 sec) + Echoic (auditory, ~3-4 sec)
 ↓ attention
 SHORT-TERM / WORKING MEMORY (~30 sec without rehearsal)
 Capacity $\sim 7 \pm 2$ items. Chunking expands capacity.
 ↓ encoding
 LONG-TERM MEMORY (effectively unlimited, lasts indefinitely)
 Explicit (declarative): semantic (facts) + episodic (events) – hippocampus
 Implicit (procedural): skills + classical conditioning – cerebellum/basal ganglia

Encoding [3]/[4]

- **Levels of processing [4]:** Deeper (semantic) processing → better memory than shallow (phonemic, structural).
- **Self-reference effect [4]:** info about oneself remembered best.
- **Spacing effect [4]:** spaced practice beats massed ("cramming").
- **Mnemonics [3]:** memory aids (acronyms, method of loci, peg system).

Forgetting [3]/[4]

- **Encoding failure [3]:** never made it to LTM.
- **Storage decay [3]:** Ebbinghaus forgetting curve — rapid loss, then leveling off.
- **Retrieval failure [3]:** info there but inaccessible. Tip-of-the-tongue phenomenon.
- **Proactive interference [4]:** old memories interfere with new (called wrong name, used to ex).
- **Retroactive interference [4]:** new memories interfere with old (forgot French after learning Spanish).
- **Misinformation effect [4]:** memory altered by post-event misleading info (Loftus).

KEY INSIGHT [5] Memory is **RECONSTRUCTIVE**, not video playback. Each retrieval can subtly alter the memory. This has huge implications for eyewitness testimony, recovered memories, and autobiographical accuracy.

Thinking & Decision-Making [4]

- **Algorithm:** step-by-step, guaranteed solution (slow but reliable).
- **Heuristic:** mental shortcut (fast but error-prone).
- **Availability heuristic [4]:** judging frequency by how easily examples come to mind. Why people fear plane crashes more than car accidents.
- **Representativeness heuristic [4]:** judging by similarity to prototype. Source of stereotype-based errors.
- **Confirmation bias [3]:** seeking info that confirms beliefs.
- **Framing [4]:** how info is presented affects choice ("90% survival" vs "10% mortality").

Intelligence [3]/[4]

- **Spearman's g [4]:** general intelligence factor.
- **Gardner's multiple intelligences [4]:** 8 types (linguistic, logical-mathematical, spatial, musical, bodily-kinesthetic, interpersonal, intrapersonal, naturalistic).
- **Sternberg's triarchic [4]:** analytical + creative + practical.
- **IQ formula:** mental age / chronological age × 100. Modern: deviation IQ; mean = 100, SD = 15.
- **Reliability vs validity [3]:** consistent results vs measuring what it claims to measure.

UNIT 3 | DEVELOPMENT & LEARNING · conditioning is everything

Classical Conditioning (Pavlov) [3]

Before conditioning: UCS (food) → UCR (salivation) [reflex, no learning]
 NS (bell) → no salivation

During conditioning: NS (bell) + UCS (food) → UCR (salivation) [pair repeatedly]

After conditioning: CS (bell) → CR (salivation) [now bell triggers salivation]

KEY PROCESSES:

- Acquisition — initial learning
- Extinction — CR fades when CS no longer paired with UCS
- Spontaneous recovery — extinguished CR reappears
- Generalization — similar stimuli also trigger CR
- Discrimination — only specific CS triggers CR

Operant Conditioning (Skinner) [3]/[4]

Behavior is shaped by its consequences. Reinforcement INCREASES behavior; punishment DECREASES it.

Type	Definition	Example
Positive reinforcement [3]	ADD desired stimulus to INCREASE behavior	Treat for sitting
Negative reinforcement [3]	REMOVE aversive stimulus to INCREASE behavior	Seatbelt beep stops when buckled
Positive punishment [3]	ADD aversive stimulus to DECREASE behavior	Spanking, scolding
Negative punishment [3]	REMOVE desired stimulus to DECREASE behavior	Time-out, taking away phone

Schedules of Reinforcement [4]

- **Continuous reinforcement** — every response reinforced. Fast learning, fast extinction.
- **Fixed-Ratio (FR)** — reinforce after fixed # of responses. Pause after reward.
- **Variable-Ratio (VR)** — reinforce after variable # of responses. **Most resistant to extinction.** Gambling, fishing.
- **Fixed-Interval (FI)** — reinforce first response after fixed time. Scallop pattern (slow then fast as time approaches).
- **Variable-Interval (VI)** — reinforce first response after variable time. Steady, moderate response rate.

Observational Learning [3]

Bandura's Bobo doll experiment: children who watched adults aggress against doll were more aggressive. **Mirror neurons** thought to be neural basis.

Lifespan Development — key stages [3]/[4]

- **Piaget's cognitive stages [4]:** Sensorimotor (object permanence ~8 mo) → Preoperational (egocentrism, no conservation) → Concrete operational (logic w/ concrete objects) → Formal operational (abstract reasoning ~12).
- **Erikson's psychosocial stages [4]:** 8 stages, each a crisis. Trust vs mistrust (infancy); Autonomy vs shame; Initiative vs guilt; Industry vs inferiority; Identity vs role confusion (adolescence); Intimacy vs isolation; Generativity vs stagnation; Integrity vs despair.
- **Kohlberg's moral development [5]:** Pre-conventional (avoid punishment, self-interest) → Conventional (social approval, law/order) → Post-conventional (social contract, universal ethics).
- **Attachment styles [4]:** Secure, anxious-resistant, anxious-avoidant, disorganized. Studied by Ainsworth's Strange Situation.

KEY INSIGHT [5] Nature vs nurture is FALSE DICHOTOMY. Modern view: gene × environment interaction. Genes don't determine outcomes alone — they shape sensitivity to environment. Twin and adoption studies tease apart contributions.

UNIT 4 | SOCIAL PSYCHOLOGY & PERSONALITY

Social Influence — pressure of others [3]/[4]

- **Conformity [3]** — Asch line studies. ~37% conform even when answer is obviously wrong.
- **Normative social influence [4]** — conform to fit in (avoid rejection).
- **Informational social influence [4]** — conform because we think others know better.
- **Obedience [3]** — Milgram shock studies. 65% delivered max shock when ordered. Obedience higher when: authority figure present, victim distant, no role models who refused.
- **Group polarization [4]** — group discussions amplify members' initial views.
- **Groupthink [4]** — striving for consensus suppresses dissent. Janis: Bay of Pigs, Challenger disaster.
- **Bystander effect [3]** — Darley & Latané. More bystanders → less help. Diffusion of responsibility.

Attribution & Attitudes [3]/[4]

- **Fundamental attribution error [3]** — overestimate dispositional (personality) factors, underestimate situational factors when judging OTHERS.
- **Self-serving bias [4]** — attribute successes to self, failures to circumstances.
- **Cognitive dissonance [3]** — discomfort when attitudes and behavior conflict. Resolved by changing attitude (Festinger "\$1 vs \$20" study).
- **Foot-in-the-door [4]** — small request first, big request later (more compliance).
- **Door-in-the-face [4]** — big request first (rejected), small request second (accepted).

Personality Theories [4]/[5]

Theory	Key idea
Freud's psychoanalytic [4]	Id (pleasure) vs Ego (reality) vs Superego (morality). Defense mechanisms hide unconscious conflicts.
Neo-Freudians [5]	Adler (inferiority), Jung (collective unconscious), Horney (basic anxiety)
Humanistic (Rogers, Maslow) [4]	People are inherently good; strive for self-actualization. Need unconditional positive regard.
Trait theory: Big Five (OCEAN) [4]	Openness, Conscientiousness, Extraversion, Agreeableness, Neuroticism. Genetic + environmental influence.
Social-cognitive (Bandura) [4]	Reciprocal determinism: personality, environment, and behavior all influence each other. Self-efficacy.
Behavioral [4]	Personality is learned through reinforcement history.

Defense Mechanisms (Freud) [4]

- **Repression** — pushing threatening thoughts out of consciousness.
- **Denial** — refusing to accept reality.
- **Projection** — attributing one's unwanted feelings to others.
- **Rationalization** — making excuses.
- **Displacement** — redirecting feelings to safer target (yelling at family after bad day at work).
- **Sublimation** — channeling impulses into socially acceptable activities.
- **Reaction formation** — expressing opposite of what one feels.

Personality Assessment [4]

- **Projective tests [4]:** TAT, Rorschach inkblots. Assume people project unconscious onto ambiguous stimuli. **Low reliability + validity.**
- **Self-report inventories [4]:** MMPI, NEO-PI. More reliable + valid than projective. Vulnerable to faking.

UNIT 5 | MENTAL & PHYSICAL HEALTH · disorders + treatments

Stress & Coping [3]/[4]

- **Stress** = perceived threat to wellbeing requiring adaptation.
- **Selye's GAS [3]**: Alarm (fight-or-flight) → Resistance (sustained mobilization) → Exhaustion (resources depleted).
- **Stress effects [4]**: suppresses immune system; risks cardiovascular disease; affects telomere length.
- **Coping strategies [4]**: Problem-focused (change the situation) vs Emotion-focused (regulate response).

Major Disorders — DSM categories [3]/[4]

Disorder category	Examples	Key features
Anxiety disorders [3]	GAD, panic disorder, specific phobias, agoraphobia	Excessive worry/fear; physical arousal symptoms
 OCD [3]	Obsessive-compulsive disorder	Intrusive thoughts (obsessions); repetitive behaviors (compulsions) to reduce anxiety
PTSD [3]	Post-traumatic stress disorder	After trauma: flashbacks, nightmares, hypervigilance, avoidance
Mood disorders [3]	Major depressive disorder, bipolar disorder	Persistent sadness/hopelessness (MDD); alternating mania + depression (bipolar)
Schizophrenia [4]	Schizophrenia spectrum disorders	Positive symptoms (hallucinations, delusions, disorganized speech); Negative (flat affect, social withdrawal)
Dissociative [4]	Dissociative identity, amnesia, fugue	Disruption of memory, identity, or perception
Personality disorders [4]	Borderline, antisocial, narcissistic, OCPD	Persistent maladaptive personality patterns
Neurodevelopmental [4]	ADHD, autism spectrum disorder	Onset in childhood; affects functioning across domains
Eating disorders [4]	Anorexia, bulimia, binge eating	Disturbance in eating behavior, body image

Theories of Disorder Causes [4]

- **Biological [3]**: genes, neurotransmitters, brain structure. Depression linked to low serotonin; schizophrenia to excess dopamine.
- **Cognitive [4]**: distorted thinking patterns. Beck's depression triad (negative views of self, world, future).
- **Behavioral [4]**: maladaptive learning. Phobias from classical conditioning.
- **Diathesis-stress model [5]**: genetic vulnerability + environmental stressor → disorder.

Therapies [3]/[4]

Therapy	Approach
Psychoanalysis [4]	Freudian. Free association, dream analysis, transference. Goal: bring unconscious to consciousness.
Cognitive therapy [4]	Beck, Ellis. Identify and challenge distorted thoughts.
Behavioral therapy [4]	Apply conditioning principles. Systematic desensitization for phobias (graded exposure + relaxation).
Cognitive-Behavioral Therapy (CBT) [3]	Most evidence-based. Combines cognitive + behavioral techniques. Effective for many disorders.
Humanistic (Rogers) [4]	Client-centered. Active listening, unconditional positive regard, genuineness.
Group therapy [4]	Multiple clients with shared issues. Cheaper, social support.
Biomedical [3]	Drugs + medical interventions. SSRIs (depression), antipsychotics (schizophrenia), benzodiazepines (anxiety), lithium (bipolar). Also ECT, TMS.

KEY INSIGHT [5] **Therapy effectiveness research**: psychotherapy works (esp. CBT). Drug + therapy combination often best for severe disorders. Therapeutic alliance (relationship with therapist) is one of the strongest predictors of success.

PAGE 9 | FAMOUS STUDIES & RESEARCHERS · the names + studies the exam loves

These classic studies appear in MCQ scenarios constantly. Knowing the researcher + study + finding earns easy points and helps you recognize hypothetical scenarios on FRQs.

Researcher / Study	What they did	Findings + tier
Pavlov (1900s)	Conditioned dogs to salivate at bell sound by pairing it with food	Founded classical conditioning . UCS, UCR, CS, CR. [3]
Watson (Little Albert, 1920)	Conditioned 11-mo-old to fear white rat by pairing with loud noise	Showed fear can be conditioned in humans . Generalized to all white furry things. Ethically problematic. [3]
Skinner (1930s+)	Operant conditioning chambers ("Skinner boxes") with rats and pigeons	Operant conditioning . Reinforcement schedules. Behavior shaped by consequences. [3]
Bandura (Bobo doll, 1961)	Children watched adults aggress against inflatable doll	Observational learning . Children imitated aggression even without direct reinforcement. [3]
Asch (1951)	Asked subjects to identify line lengths in groups (confederates gave wrong answer)	~ 37% of trials , subjects conformed to obviously wrong group answer. Conformity . [3]
Milgram (1961)	Subjects ordered to give increasingly painful shocks to confederate	65% delivered max shock (450V) under authority pressure. Obedience to authority. [3]
Zimbardo (Stanford Prison, 1971)	College students assigned to play guards or prisoners	Guards quickly became abusive; study terminated early. Power of social roles + situations . Ethically controversial. [4]
Festinger & Carlsmith (1959)	Subjects paid \$1 or \$20 to lie about a boring task	\$1 group reported task was MORE enjoyable. Cognitive dissonance — lower payment couldn't justify the lie, so attitude changed. [4]
Loftus (1970s+)	Eyewitnesses asked leading questions about car accident	Wording ("smashed" vs "hit") changed memory. Misinformation effect . Eyewitness testimony unreliable. [3]
Harlow (Monkeys, 1958)	Baby monkeys with wire vs cloth surrogate mothers	Monkeys preferred CLOTH mother despite wire mother providing food. Contact comfort > nutrition for attachment. [4]
Ainsworth (Strange Situation)	Observed infant reactions to mother leaving + returning	Identified secure, anxious-resistant, anxious-avoidant attachment styles. [4]
Piaget (1920s+)	Studied children's cognitive development	4 cognitive stages . Object permanence, conservation, egocentrism. [3]
Ebbinghaus (1885)	Memorized nonsense syllables, tested recall over time	Forgetting curve : rapid initial loss, then leveling off. Spacing effect discovered later. [4]
Rosenhan ("On Being Sane in Insane Places", 1973)	Healthy volunteers admitted to psychiatric hospitals	All diagnosed with schizophrenia despite normal behavior. Diagnostic labels are powerful . Critique of psychiatric diagnosis. [4]
Seligman (Learned helplessness)	Dogs given inescapable shocks didn't try to escape later	Learned helplessness . Model for depression . [4]
Maslow	Studied self-actualizing people	Hierarchy of needs . Humanistic psychology emphasizes growth + potential. [3]
Genie ("Wild Child", 1970)	Girl isolated until age 13 with no language exposure	Could not learn full language. Supports critical period hypothesis for language. [4]

WATCH OUT [3] **Common MCQ trap**. Mixing up similar names: **Skinner** vs **Pavlov** (operant vs classical); **Piaget** vs **Erikson** (cognitive vs psychosocial stages); **Watson** (Little Albert) vs **Bandura** (Bobo doll). Drill the differences.

PAGE 10 | FRQ STRATEGY · the new AAQ + EBQ format (post-2024)

The 2024 AP Psych redesign created TWO new FRQ types, replacing the old free-response questions. Both require analyzing source material.

FRQ 1 — Article Analysis Question (AAQ) [3]/[4]

Format: You read a brief description of a published study, then answer questions about its design, results, and conclusions. **Worth ~7 points.**

Typical sub-questions:

- [3] **Identify** the IV and DV.
- [3] **Identify** a strength or limitation of the methodology.
- [4] **Apply** a psychological concept to the study's findings.
- [4] **Explain** how the findings support or refute a theory.
- [5] **Evaluate** the validity of generalizing the findings to a broader population.

FRQ 2 — Evidence-Based Question (EBQ) [4]/[5]

Format: You're given a research question + 3 sources (articles, summaries). Develop a defensible **claim** + use evidence from at least 2 sources to support. **Worth ~7 points.**

Required elements:

- [3] **Defensible claim** that responds to the prompt
- [4] **Use evidence from at least 2 of the 3 sources**
- [4] **Reasoning** explaining how evidence supports the claim
- [5] **Acknowledge counterevidence or alternative explanation**

UNIVERSAL FRQ DISCIPLINE [4]/[5]

- [3] **Use psychology terms**, not casual language. "Operant conditioning" not "learning by reward."
- [3] **Apply concepts to the SCENARIO** in the prompt — don't just define.
- [4] **Cite the perspective by name** when relevant: "From a behavioral perspective..."
- [4] **When discussing studies**, name the researcher: "Similar to **Milgram's** obedience study..."
- [5] **Sophisticated argumentation:** acknowledge limits, generalizability, alternative interpretations.

FRQ ANSWER TEMPLATE [4]

Step 1: STATE the concept/perspective ("Operant conditioning involves...")
 Step 2: CONNECT to the scenario ("In this study, the researcher reinforced...")
 Step 3: EXPLAIN the implication ("This means subjects would learn to...")
 Step 4: [5] EVALUATE – acknowledge limit or alternative explanation

Pattern: STATE → CONNECT → EXPLAIN → (EVALUATE for 5)

PAGE 11 | THE 3/4/5 SCORE LADDER · tier-specific test-day

THE SCORE LADDER — same prompt, three tiers of answer

3-scorer writes...	4-scorer writes...	5-scorer writes...
"The kid learned not to do it because of punishment."	"The child learned not to do it through positive punishment — adding an aversive stimulus to decrease behavior."	"The child learned to suppress the behavior through positive punishment , where adding an aversive stimulus (scolding) decreased future occurrence. From a behavioral perspective , this aligns with Skinner's operant conditioning. However, [5] punishment can produce side effects like fear of the punisher rather than internalized rule-following — this is why behaviorists often recommend reinforcing alternative behaviors instead."
"It's like the Asch study."	"This resembles Asch's conformity studies, where ~37% conformed to wrong answers under group pressure."	"This parallels Asch's conformity studies. However, the comparison has limits — Asch's task was unambiguous (line length), so conformity was normative (fitting in). The current scenario involves an ambiguous task, which would activate informational social influence (others as info source). Both forms produce conformity but through different mechanisms."
"He has depression."	"His symptoms (persistent sadness, loss of interest, sleep issues) suggest major depressive disorder ."	"His symptoms map onto DSM criteria for major depressive disorder. From a biological perspective , this might involve serotonin dysregulation (consistent with SSRI effectiveness). From a cognitive angle (Beck), he may have a negative cognitive triad. Best treatment: CBT combined with SSRIs has the strongest evidence base."
Identifies IV and DV	Identifies IV/DV + a strength or limitation of the design	Adds: alternative explanations + generalizability concerns + how findings connect to broader theory

TIER-SPECIFIC TEST-DAY CHECKLISTS

For a 3 — bag ALL of these:

- ✓ MCQ: 38+ correct. Don't leave anything blank.
- ✓ Memorize the 5 perspectives + Famous Studies (page 9).
- ✓ Each FRQ part: at least define the concept and apply it loosely to the scenario.
- ✓ Identify IV/DV in research scenarios.
- ✓ Distinguish classical vs operant conditioning. Memorize the four reinforcement/punishment cells.

For a 4 — add these on top:

- ✓ MCQ: 46+ correct. Drill subtle distinctions (proactive vs retroactive interference; FR vs VR schedules).
- ✓ FRQ: name the perspective explicitly. Cite a researcher when relevant.
- ✓ For research scenarios: identify both strengths AND limitations of design.
- ✓ Apply DSM criteria to clinical scenarios.
- ✓ Distinguish nature vs nurture as a FALSE dichotomy — most behavior is gene × environment.

For a 5 — add these on top:

- ✓ MCQ: 56+ correct.
- ✓ FRQ: cite multiple perspectives + acknowledge alternative interpretations.
- ✓ Discuss limits to generalizability (sample size, demographics, cultural context).
- ✓ EBQ: use evidence from all 3 sources. Address counterevidence.
- ✓ Use technical language consistently — DSM categories, reinforcement schedules, neurotransmitter names.
- ✓ Connect findings to broader theory (e.g., "This supports cognitive theory because...").

THE ONE THING THAT MATTERS MOST [3][4][5]: APPLY THE CONCEPT TO THE SCENARIO. AP Psych FRQs always involve a hypothetical situation. Generic definitions earn no points; you must **SHOW** how the concept manifests in the specific scenario described. "Schema is X" earns 0; "Schema is X — and in this scenario, Maria's schema for restaurants causes her to expect a hostess upon entering, leading her to feel confused when there isn't one" earns the point.

If you do nothing else: study to your tier (badges), apply concepts to scenarios on FRQs, and name perspectives + researchers explicitly. That's your score.

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